



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : نجوى امام امام على
المحافظة : القاهرة الحي / المدينة : حلوان
رقم البطاقة : 26010070103668 رقم التليفون : 01092991846
العنوان : حلوان
العمر (تاريخ الميلاد) : 07-10-1960 المهنة : حاصل علي دبلوم
تاريخ تسجيل الإستمارة : 04-10-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....) المقر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام
التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: نجوى امام امام علي
National ID: 26010070103668



تقرير طبي

المسيد

تحية طيبة وبعد,,

بشان تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بان نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الطول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالبدی و تحتاج الي Further investigations and imaging

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



لجوى
امام امام على ابراهيم
٢١ ش مصطفى فهمي
خلوان - القاهرة

١٩٩٠/١١/٢٧ ٢٦٠١٠٠٧٠١٠٣٦٦٨

IJ4511938

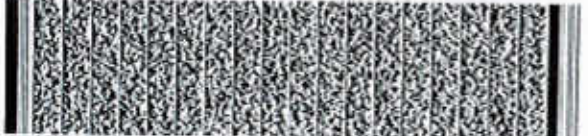
٢٠٢٠/١٠/٢٦ ٢٦٠١٠٠٧٠١٠٣٦٦٨

حاصلة على الثانوية عامة

أنش مسجلة
سمير سيد محمد حامد

البطاقة سارية حتى ٢٠٢٧/١٢/٢٦





04/16/2026

2

MDT FORM

Name:	Nagwa Imam Imam Aly	HRN:	256
Age:	65 Y	ID:	26010070103668
Menopausal Sta:	POST	Phone:	01092991846
NRS SCORE:		Marital Status:	
BMI:		Family History:	ve her sister +
CO Morbidity:	hln .DM	Reason Of MDT:	post op

Clinical Data:

BX: IDC G11
ER: 8/8/ PR: 8/8 HER2: +ve KI67:

R

L

Sonomammography:

Meta Workup:

Bil small pulmonary nodules

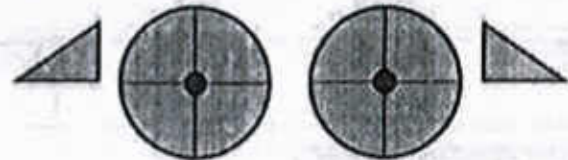
History:

post nact ptn underwent RT HRM
post op pathology
residual viable IDC G11
DCIS
ALND : (9/10) with extranodal extension
ypT3.ypN3

Recommendations:

MDT Outcome & Plan

High risk case CT4bN2 (initial CT CAP showed small pulmonary nodules, IN and mediastinal LNs, HFLs and Bone sclerotic lesions... none of them proved to be metastatic but for dose FU).
NACT and Rt HRM --> ypT3N3 for: -Adj
1/kadcyla 12 cycles
2/ET Femara (7y) + Zometa/6m
3/PORT to CW + LNs (inc IMC)
4/FU by CT CAP e contrast now and after 6m



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/معتز حوت	د/اسماء محمد				

Name: نجوى امام علي		GUSTAVE ROUSSY
MR. : 256		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient is a 65-year-old individual with HER2-positive breast cancer. Initial neoadjuvant chemotherapy consisted of AC*4 followed by 4 cycles of taxane (TXT) and 3 cycles of PHESGO, with the last PHESGO cycle administered on 25-Jan-2026. Subsequent imaging revealed CT4B disease with N2 nodal involvement and pulmonary nodules, and the patient was noted to have an ER8/8, PR8/8, HER2 SISH amplified tumor. The plan included AC, 4 cycles of taxotere, and 3 cycles of PHESGO, with surgery pending to assess pulmonary nodules and a left ventricular ejection fraction of 64% as of 28-Jan-2026. The patient underwent right mastectomy (RM) on 28-Feb-2026 following the completion of PHESGO cycle 5, with postoperative multidisciplinary team review and an EF of 64% noted on 28-Jan-2026. On 07-Jun-2026 the patient received radiotherapy to the left breast (RT MEM) on 28-Feb-2025, with a plan for a new CT port and initiation of Femara and Kadcylla therapy. On 28-Jul-2026 a CT scan of the chest, abdomen, and pelvis was performed, showing a 2-3 mm pulmonary nodule, an 18 × 15 mm splenic hypodense lesion, a small sclerotic lesion at L3, and HFLs measuring 18 × 15 mm. The patient received Femara only today for dynamic MRI over the liver.		
تحريراً في ,		
	التاريخ :	19-09-2026
	الوقت :	dina makram
الطبيب المعالج :		
يعتمد ,		

Patient Name : الأستاذ / نجوي امام امام علي	Request No. : 13005242
Patient No. : 26010070103668	Request Date : 28/02/2026
Path No. : 574/26	Verification Date : 08/03/2026
Sex : Female	Financial Category : جوسف روسي - نفقة الدولة
Age : 65 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast cancer, Post-NACT.
Nature of Specimen: Right modified radical mastectomy.

Gross Examination:

- * Right modified radical mastectomy specimen measured 25x15x5 cm, with covering skin ellipse measured 22x14 cm. Nipple & areola looked unremarkable grossly.
- * Serialing showed a tumor bed site measured 11x8x3 cm, located 2 cm away from the deep margin.
- * Axilla: Axillary fat measured 8x7x2 cm, entangling 10 lymph nodes, the largest was 1.5x1x0.5 cm.
- 17 Blocks:
- 1:11- T.B.S., 12- N&A., 13- Deep margin., 14:17- ALNs.
- Gross was done by; Dr. Heba Hassan.

Microscopic examination:

- * Histological type: Residual viable invasive duct carcinoma. (Actual tumour size 6x5 cm) (Cellularity 40%).
- * Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
- * Ductal carcinoma in situ: 20% Intermediate grade DCIS (Solid, papillary & cribriform pattern).
- * Angioinvasion: Present.
- * Perineural invasion: Absent.
- * Microcalcification: Present.
- * Nipple and areola: Free.
- * Deep margin: Free.
- * Axillary lymph nodes: Positive viable metastatic deposits in nine out of ten lymph nodes (9/10) (The largest metastatic focus measuring 1 cm, with extra-nodal extension).
- * Histologic response to therapy: Pathologic partial response to therapy.

Verified by :
Dr.Duaa Mohamed



Patient Name : الأستاذة / نجوي امام علي
Patient No. : 26010070103668
Path No. : 574/26
Sex : Female
Age : 65 Y
Referring Doc :

Request No. : 13005242
Request Date : 28/02/2026
Verification Date : 08/03/2026
Financial Category : جوسداف روسي - نفقة الدولة
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Diagnosis:

Right breast cancer (Post-NACT), Right modified radical mastectomy:
Residual viable invasive duct carcinoma, Grade 2 (Cellularity 40%) (Actual tumor size 6x5 cm).

- * Ductal carcinoma in situ: 20% Intermediate grade DCIS (solid, papillary & cribriform pattern).
- * Nipple and areola: Free.
- * Deep margin: Free.
- * Axillary lymph nodes: Positive viable metastatic deposits in nine out of ten lymph nodes (9/10) (The largest metastatic focus measuring 1 cm, with extra-nodal extension).
- * Stage: ypT3, ypN3a.
- * RCB: 4.344, RCB class-III.

Dr. Walaa Ibrahim Ghoneim.

Signature

Verified by :
Dr.Duaa Mohamed



MDT

Patient Name	دیدی امام بختی	Age	65
Sex	Female	Special habits	
Family History	VE	Co-morbidity	MEDICAL:DM,H1N
Surgical History		Telephone	01092991846
Menopausal Status	POSTMENOPAUSAL	Marital Status	MARRIED
MEDICAL ID	2218/2025	ID	26010070103668

Pathology

RT IDC GRADE II

ER:8/8

PR:6/8

HER2:PENDING

KI67:15%

PATIENT HX

S.M

RT UOQ and retro areolar focal ASSYMMETRY OVER AREA MEASURING 8.6X3.9CM on MLO view with skin thickening and nipple retraction, diffuse oedema + level 1 pathological LNs

BENIGN LEFT BREAST FINDINGS

Stage: T4N2M0

CT CAP

LT pathological internal mammary LNs 11mm

Prominent prevascular, RT paratracheal, aortopulmonary and rt hilar LNs

Largest 1.1X1.7 cm in prevascular area

Small pulmonary nodules for follow up

BI lobar hepatic focal LESIONS MOSTV LIKELY HEMANGIOMA AND SPLENI

FOCAL LESION

RECOMMENDATIONS

Bone scan required

Slides revision

MDT outcome
and plan

For AC x 4 + 4 Taxol till sish result +/- HP if Her2 + ve
post NACT if not metastatic by new CT CAP for Rt MRM

Signature

دیدی امام بختی
20/8/19

11/8/19

Patient Name	: الأستاذ / اجري امام علي	Request No.	: 13000295
Patient No.	: 26010070103668	Request Date	: 05/08/2025
Path No.	: SR-1075/25	Verification Date	: 07/08/2025
Sex	: Female	Financial Category	: جومات روسي - نفقة الدولة
Age	: 64 Y	Department	: Out Patient
Referring Doc :		Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Right breast mass.

Nature of Specimen: Referred block & slides (Tru-cut biopsy).

Gross Examination:

One referred block labelled by (242B-25). Together with, four referred immunostained slides for (ER, PR, HER2neu & SISH) labelled by (202676-25).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade 2 (differentiation 2, nuclear pleomorphism 2, mitosis 2).
- * DCIS: Absent.
- * Angiolinvasion: Absent.
- * Perineural invasion: Absent.

- * IPT:
 - ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
 - PR: Strong nuclear reaction in 95% of tumor cells (score 8/8).
 - Her2: Intermediate incomplete membrane reaction in more than 10% of tumor cells (score 2+).
 - SISH: Amplified.
- (Average Her2 copy No.: (6.1), Her2/CEP17 ratio: (4.9)).

Diagnosis:

Right breast mass, Referred block & slides (Tru-cut biopsy):

- Invasive duct carcinoma, Grade 2.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).

Verified by :

Dr. Duaa Mohamed

Printing Date : 09/08/2025 14:46:49

Page 1 of 3

Printing By : Mrs. Mayada Hegazy



Patient Name : الأستاذ / نجوي امام علي	Request No. : 13000295
Patient No. : 26010070103668	Request Date : 05/08/2025
Path No. : SR-1075/25	Verification Date : 07/08/2025
Sex : Female	Financial Category : جنسيت روسي - نفقة الدولة
Age : 64 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

- Her2: Equivocal (score 2+).
- SISH: Amplified, Group I.
- (Average Her2 copy No.: (6.1), Her2/CEP17 ratio: (4.9)).

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

ISH groups:

The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).
- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).

Verified by :

Dr.Duaa Mohamed

Printing Date : 09/08/2025 14:46:49

Page 2 of 3

Printing By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / نجري امام علي

Patient No. : 26010070103668

Path No. : SR-1075/25

Sex : Female

Age : 64 Y

Referring Doc :

Request No. : 13000295

Request Date : 05/08/2025

Verification Date : 07/08/2025

Financial Category : نفقة الدولة - جوسفات روسي

Department : Out Patient

Registered by : marwa mohamed youss

Pathology Report

- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).
- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature

Dr. Walaa Ibrahim Ghonelm.

Dr. Heba Hassan Adly.

Verified by :

Dr. Duaa Mohamed

Printing Date : 09/08/2025 14:46:49

Page 3 of 3

Printing By : Mrs. Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



SCAN & LABS

Name : Nagwa Iniam Iniam Ali

Branch : Dokki

مركز النيل للأشعة والتحاليل

Date : 07-08-2025

Medical No. : 170188

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History:

- 65-year-old female, with right breast cancer, referred for metastatic workup.

Technique:

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings:

- No abnormal active or cold focal bony lesions that may account for metastatic bone disease could be seen all over the surveyed skeleton.
- Degenerative changes are seen at the spine and big joints.

Impression:

- The skeleton is free from metastases.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

ECHO REPORT

Name: محمد صالح

Date: 14/8/2025

EF	60%	55-75%	PWT	0.9	0.6-1.1cm
LVEDD	4.5	3.5-5.8cm	AO.R	2.3	2-3.7cm
LVEDS	2.8	2-4cm	LA	3.1	2-4cm
FS	28%	25-45%	RV		0.7-2.3cm(supine)
SWT	0.8	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV dimensions Normal

LV wall thickness Normal

LV diastolic function GI

SWMA No RWMA

LA and Ao root dimensions Normal

Mitral Valve ✓

Aortic valve ✓

Tricuspid valve ✓

Pulmonary valve

Right side and systolic function TAPSE: 2.8

Intracardiac masses and effusion

Dr / محمد صالح

PATIENT NAME: NAGOWA EMAM EMAM ALI

GENDER: Female

DATE : 20-Jul-2025 02:14 PM

PATIENT ID: 20225021411

AGE: 64 y

ORDER: Z222005,Z222062

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITH TRIPHASIC CT STUDY ASESSMENT OF THE LIVER

TECHNIQUE:

- Multislice helical CT acquisition with automatic IV low osmolar non-ionic contrast medium infusion 70 ml.

FINDINGS:

****Reported case of breast cancer.**

1-CHEST

- Right breast locally advanced neoplastic process in the form of ill defined enhancing confluent soft tissue lesions, the largest lesion, being retroareolar in location, measuring about 3 x 4.1 cm in maximum axial dimensions, being inseparable from the nipple/areola complex. Overlying diffuse skin thickening. Associated parenchymal edema is noted as well.
- Multiple pathologically enlarged discrete and partially amalgamated right level I and II axillary lymph nodes, the largest is seen at level I, measuring about 1.5 x 2.5 cm in maximum axial dimensions. Stranding of the related fat planes is noted.
- Pathologically enlarged right internal mammary lymph nodes, the largest measures about 0.6 x 1 cm in maximum axial dimensions.
- Enlarged left internal mammary lymph node is seen measuring 11 mm.
- Few small bilateral pulmonary nodules, the largest is seen at the left lower lung lobe, measuring about 0.6 cm in diameter.
- No pulmonary masses, areas of consolidation or cavitary lesions.
- Prominent prevascular, right paratracheal, aortopulmonary and right hilar lymph nodes, the largest is seen at the prevascular group, measuring about 1.1 x 1.7 cm in maximum axial dimensions.

PATIENT: NAGOWA EMAM EMAM ALI, 20225021411, Female, 64 y
Report version 2, 21-Jul-2025 11:39 AM



- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- Minimal pericardial effusion.

• ABDOMEN AND PELVIS

- The liver is enlarged with homogenous CT texture, showing few small bilobar hepatic focal lesions, the largest is seen at the right hepatic lobe segment VI, measuring about 0.8 cm in diameter showing enhancement in the arterial phase and became isodense to the liver parenchyma in the portal and delayed phases of the study.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- Mild dilatation of the CBD (reaching 1.2 cm in diameter) and central intrahepatic biliary radicles likely representing post cholecystectomy sequel.
- Average sized spleen showing inferior pole focal lesion, measuring about 1.4 x 1.6 cm in maximum axial dimensions, that appear hypodense to the splenic parenchyma in the arterial and portal phases of the study and became isodense to the splenic parenchyma in the delayed phase of the study.
- Mildly dilated right pelvicalyceal system with abrupt changes of caliber at the pelvi-ureteric junction, likely pelvi-ureteric junction obstruction. Prominent left pelvicalyceal system.
- Right adrenal soft tissue nodule, measuring about 0.9 x 1.3 cm in maximum axial dimensions.
- Normal appearance of the pancreas, left adrenal, aorta (apart from aortoiliac atheromatous calcifications) and I.V.C.
- Partial filling of the urinary bladder with no filling defects or diverticular outpouching.
- No pathologically enlarged abdominal or pelvic lymph nodes.
- IUD is noted in situ with distended endocervical canal by hypodensity, for sonographic correlation.
- Small left adnexal cyst, measuring about 0.8 cm in diameter.
- No abdominal or pelvic masses or collections.
- The revised bone window revealed:
- Osteopenic texture of the examined bones.
- Ventral wedging of L4 vertebral body with sagging depression of its superior vertebral endplate.
- A sclerotic area is seen involving L3 vertebral body.
- Healing fracture of the anterolateral aspect of the right 8th rib with related callus formation.
- Small sclerotic area at the anterolateral aspect of the right 9th rib.
- Diffuse osteopenic texture of the examined bones is noted.

PATIENT NAME: NAGOWA EMAM EMAM ALI

GENDER: Female

DATE : 20-Jul-2025 02:14 PM

PATIENT ID: 20225021411

AGE: 64 y

ORDER: Z222005,Z222062

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITH TRIBASIC CT STUDY ASEESSMENT OF THE LIVER

TECHNIQUE:

- Multislice helical CT acquisition with automatic IV low osmolar non-ionic contrast medium infusion 70 ml.

FINDINGS:

****Reported case of breast cancer.**

1-CHEST

- Right breast locally advanced neoplastic process in the form of ill defined enhancing confluent soft tissue lesions, the largest lesion, being retroareolar in location, measuring about 3 x 4.1 cm in maximum axial dimensions, being inseparable from the nipple/areola complex. Overlying diffuse skin thickening. Associated parenchymal edema is noted as well.
- Multiple pathologically enlarged discrete and partially amalgamated right level I and II axillary lymph nodes, the largest is seen at level I, measuring about 1.5 x 2.5 cm in maximum axial dimensions. Stranding of the related fat planes is noted.
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- Prominent prevascular, right paratracheal, aortopulmonary and right hilar lymph nodes, the largest is seen at the prevascular group, measuring about 1.1 x 1.7 cm in maximum axial dimensions.

PATIENT: NAGOWA EMAM EMAM ALI, 20225021411, Female, 64 y
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Page 1 of 3



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16602

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في شارع طوبى قول شارع الدم بجوار دوى، الجيزة



مستشفى بيهية الشيخ زايد
في الحي الأول - الشرق زايد امام كلية الهندسة



PATIENT NAME: NAGOWA EMAM EMAM ALI
GENDER: Female
DATE : 20-Jul-2025 12:44 PM

PATIENT ID: 20225021411
AGE: 64 y
ORDER: Z222002,Z222003

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:

- Pathologically proven right breast UOQ & retro-areolar lesions (invasive duct carcinoma grade II) done outside Bahyea.

TECHNIQUE:

- CC and MLO view.
- Complementary ultrasound

REPORT:

- Scattered fibroglandular densities (ACR B).

Right breast:

- UOQ and retro-areolar focal asymmetry over an area measuring about 8.6 x 3.7 cm on MLO view with overlying skin thickening and nipple retraction. This was confirmed by Complementary Ultrasound which showed opposite 10 to 1 O'clock diffuse edema pattern in the form of thickened skin (reaching 5mm), interstitial edema and echogenic fat lobules along with multiple irregular hypoechoic spiculated vascular masses, the largest measures about 6 x 2.4 cm and about 0.6cm from overlying skin and seen reaching g the nipple causing its retraction.
- Pathological axillary and level I LNs with largest seen axillary in location measuring about 1.7 x 2.3 cm with cortical thickness reaching 9mm.

Left breast:

- No spiculated mass lesions or grouped microcalcifications.
- This was confirmed by Complementary Ultrasound which showed no solid or cystic mass lesions.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Nonspecific axillary lymph nodes.

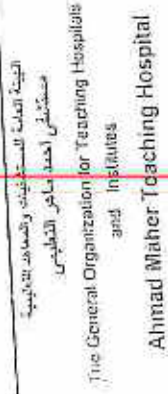
OPINION:

- Right breast UOQ and retro-areolar pathologically proven IDC with pathological axillary LNs as described (BIRADS-6).
- Benign left breast findings (BIRADS-2).

RECOMMENDATION:

Referral to clinic





Mrs. / Mr. Subhash Chandra, 65 years old
female/ male patient presented with diagnosis Rt. Breast ID
She/ He was referred to us for (radical / adjuvant / Neoadjuvant /
palliative) Radiotherapy
She / He has received (3D / IMRT) radiotherapy to Chest Wall + PCLMS
+ Breast 2st 7 sessions

At Dose : 267 Gyf. : 16 Ex. 3 w.
Additional (Boost) : yes / no at dose Gyf. Ex. w.
(sequential / concomitant)

Started: 6/8/2026.
 Ended: 7/9/2026. With good tolerance.

Head of department
Dr. Henry William